



CAREORBIT ASSESS · PUBLISHED VA EVIDENCE

eScreening across the VA: the engine behind Assess, *tested in the open literature*

The assessment approach behind CareOrbit Assess was developed at the VA in 2012 and studied in three peer-reviewed publications: a 1,372-veteran comparison against paper screening, a four-clinic implementation study, and a veteran satisfaction study. These are VA program results, cited as evidence for the approach Assess is built on.

TOPLINE RESULTS · ELECTRONIC SCREENING AGAINST PAPER · PITTMAN ET AL., 2017

10.5 days

FASTER INTO THE RECORD

Median time from screening to record entry fell from 10–11 days on paper to 1 day.

98–100%

SCREENS COMPLETED

TBI, PTSD, depression, and alcohol screens documented at 98.4–99.7%, vs. 89–93% on paper.

5×

FASTER FOLLOW-UP

Median 1 business day from screening to Care Management follow-up, vs. 5 on paper.

19+

VA MEDICAL CENTERS

Adopted by 2022 through VA Diffusion of Excellence, 18 more in progress.

THE FULL LEDGER

What the published studies measured

SPEED TO THE RECORD

Screening to medical record documentation

1 day

median with eScreening

10–11

days median on paper

Across four mandated clinical reminders, results moved into the record the same or next day — an average median saving of 10.5 days per screen. (Pittman 2017)

COMPLETION

Documentation of mandated health screens

98–100%

with eScreening

89–93%

on paper

Six-month chart review of TBI, PTSD, depression, and alcohol screens. Depression documentation reached 99.7% vs. 93.0% on paper. (Pittman 2017)

CONNECTION TO CARE

From screening into the health system

1 vs 5

business days to clinician follow-up

eScreening veterans received more specialty consults, vested into the health system faster, and had suicide risk assessments documented significantly sooner. (Pittman 2017)

SATISFACTION & DETECTION

Veteran satisfaction, and unchanged answers

Higher

satisfaction than paper

Veterans were more satisfied than with paper (2017), and highly satisfied after user-centered redesign (Almklov 2020). Rates of positive screens were comparable between media — the device did not distort what patients report.

IMPLEMENTATION STUDY

What four clinics found, including the one that said no

Pittman et al., BMC Health Services Research, 2019 — a six-month mixed-method study, cited as published, not just the flattering half.

- **One clinic fully implemented**, two partially implemented, and one did not adopt eScreening as part of normal practice after six months.
- **The technology posed no barriers** in any of the four settings, as published. The barriers were staffing support and perceived leadership support.
- **Training and technical assistance** were the important facilitators, and organizational engagement scores tracked with implementation success.
- **Why it is cited:** assessment programs succeed or fail on workflow fit, which is why Assess deploys from the workflow a system already runs rather than adding one.

WHAT PAPERWORK COSTS AT SCALE

A modeled projection, labeled as a model

An internal VA time-driven activity-based costing exercise, projected from one site's inputs — not a measured national result.

\$214M

Projected annual cost for providers and LPNs to administer questionnaires across 130 VA medical centers

910 + 650

Physician and LPN full-time equivalents projected to be consumed by questionnaire administration each year

106M

Projected physician minutes per year spent filling out questionnaires

PROVENANCE

Built by VA clinicians, spread through the VA's own vetting

- **Developed in 2012** at the VA Center of Excellence for Stress and Mental Health, VA San Diego, for the Military 2 VA care management program. Patients self-report on their own device or a clinic tablet; responses flow directly into the VA electronic medical record.
- **Named a Gold Status Practice** by the VA Under Secretary of Health in 2016 and a Diffusion of Excellence Promising Practice. Adopted at 19 VA medical centers by 2022, with 18 more in progress. Covered by US Patent 12,683,023.

THE PUBLISHED RECORD

Pittman et al., 2017, Psychological Services: 1,372-veteran comparison of eScreening against paper screening.

Pittman et al., 2019, BMC Health Services Research: mixed-method implementation study across four diverse VA clinics.

Almklov et al., 2020, Military Medicine: post-9/11 veteran satisfaction with eScreening after user-centered redesign.